

Arkansas Heart Hospital
1701 South Shackleford Road
Little Rock, AR 72211-4335

Patient: **MONSON, BERDIA T**
MRN: 00213113 Date of Service: 6/15/2026
FIN: 3670575 Provider: Paixao,MD,Andre
DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

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Education Note

Patient Education Instructions

Name: MONSON, BERDIA T Current Date: 06/18/26 10:58:25

MRN: 00213113 FIN: 3670575

The following sheet(s) are the Patient Education Leaflets for MONSON, BERDIA T

Cardiovascular

Atrial Fibrillation

Atrial fibrillation (AFib) is a type of irregular or rapid heartbeat (arrhythmia). In AFib, the top part of the heart (atria) beats in an irregular pattern. This makes the heart unable to pump blood normally and effectively.

The goal of treatment is to prevent blood clots from forming, control your heart rate, or restore your heartbeat to a normal rhythm. If this condition is not treated, it can cause serious problems, such as a weakened heart muscle (cardiomyopathy) or a stroke.

What are the causes?

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

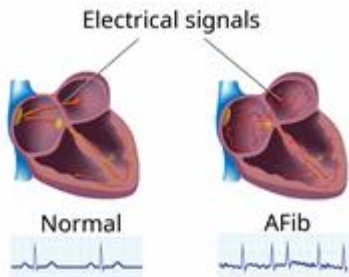
FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation



This condition is often caused by medical conditions that damage the heart's electrical system. These include:

- High blood pressure (hypertension). This is the most common cause.
- Certain heart problems or conditions, such as heart failure, coronary artery disease, heart valve problems, or heart surgery.
- Diabetes.
- Overactive thyroid (hyperthyroidism).
- Chronic kidney disease.
- Certain lung conditions, such as emphysema, pneumonia, or COPD.
- Obstructive sleep apnea.

In some cases, the cause of this condition is not known.

What increases the risk?

This condition is more likely to develop in:

- Older adults.
- Athletes who do endurance exercise.
- People who have a family history of AFib.
- Males.
- People who are Caucasian.
- People who are obese.
- People who smoke or misuse alcohol.

What are the signs or symptoms?

Symptoms of this condition include:

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- Fast or irregular heartbeats (palpitations).
- Discomfort or pain in your chest.
- Shortness of breath.
- Sudden light-headedness or weakness.
- Tiring easily during exercise or activity.
- Syncope (fainting).
- Sweating.

In some cases, there are no symptoms.

How is this diagnosed?

Your health care provider may detect AFib when taking your pulse. If detected, this condition may be diagnosed with:

- An electrocardiogram (ECG) to check electrical signals of the heart.
- An ambulatory cardiac monitor to record your heart's activity for a few days.
- A transthoracic echocardiogram (TTE) to create pictures of your heart.
- A transesophageal echocardiogram (TEE) to create even clearer pictures of your heart.
- A stress test to check your blood supply while you exercise.
- Imaging tests, such as a CT scan or chest X-ray.
- Blood tests.

How is this treated?

Treatment depends on underlying conditions and how you feel when you get AFib. This condition may be treated with:

- Medicines to prevent blood clots or to treat heart rate or heart rhythm problems.
- Electrical cardioversion to reset the heart's rhythm.
- A pacemaker to correct abnormal heart rhythm.
- Ablation to remove the heart tissue that sends abnormal signals.
- Left atrial appendage closure to seal the area where blood clots can form.

In some cases, underlying conditions will be treated.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Follow these instructions at home:

Medicines

- Take over-the-counter and prescription medicines only as told by your provider.
- **Do not** take any new medicines without talking to your provider.
- If you are taking blood thinners:
 - 46 Talk with your provider before taking aspirin or NSAIDs. These medicines can raise your risk of bleeding.
 - 46 Take your medicines as told. Take them at the same time each day.
 - 46 **Do not** do things that could hurt or bruise you. Be careful to avoid falls.
 - 46 Wear an alert bracelet or carry a card that says that you take blood thinners.

Lifestyle

- **Do not** use any products that contain nicotine or tobacco. These products include cigarettes, chewing tobacco, and vaping devices, such as e-cigarettes. If you need help quitting, ask your provider.
- Eat heart-healthy foods. Talk with a food expert (dietitian) to make an eating plan that is right for you.
- Exercise regularly as told by your provider.
- **Do not** drink alcohol.
- Lose weight if you are overweight.

General instructions

- If you have obstructive sleep apnea, manage your condition as told by your provider.
- **Do not** use diet pills unless your provider approves. Diet pills can make heart problems worse.
- Keep all follow-up visits. Your provider will want to check your heart rate and rhythm regularly.

Contact a health care provider if:

- You notice a change in the rate, rhythm, or strength of your heartbeat.
- You are taking a blood thinner and you notice more bruising.
- You tire more easily when you exercise or do heavy work.
- You have a sudden change in weight.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

Date of Service: 6/15/2026

FIN: 3670575

Provider: Paixao,MD,Andre

DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Get help right away if:

Warning signs of a stroke



Balance



Eyes



Face



Arms



Speech



Time

- You have chest pain.
- You have trouble breathing.
- You have side effects of blood thinners, such as blood in your vomit, poop (stool), or pee (urine), or bleeding that does not stop.
- You have any symptoms of a stroke. **"BE FAST"** is an easy way to remember the main warning signs of a stroke:
 - 46 **B - Balance.** Signs are dizziness, sudden trouble walking, or loss of balance.
 - 46 **E - Eyes.** Signs are trouble seeing or a sudden change in vision.
 - 46 **F - Face.** Signs are sudden weakness or numbness of the face, or the face or eyelid drooping on one side.
 - 46 **A - Arms.** Signs are weakness or numbness in an arm. This happens suddenly and usually on one side of the body.
 - 46 **S - Speech.** Signs are sudden trouble speaking, slurred speech, or trouble understanding what people say.
 - 46 **T - Time.** Time to call emergency services. Write down what time symptoms started.
- Other signs of a stroke, such as:
 - 46 A sudden, severe headache with no known cause.
 - 46 Nausea or vomiting.
 - 46 Seizure.

These symptoms may be an emergency. Get help right away. Call 911.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- **Do not wait to see if the symptoms will go away.**
- **Do not drive yourself to the hospital.**

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Heart Failure Action Plan

A heart failure action plan helps you know what to do when you have symptoms of heart failure. Your action plan is a color-coded plan that lists the symptoms to watch for and indicates what actions to take.

- If you have symptoms in the green zone, you're doing well.
- If you have symptoms in the yellow zone, you're having problems.
- If you have symptoms in the red zone, you need medical care right away.

Follow the plan that was created by you and your health care provider. Review your plan each time you visit your provider.

Green zone



These signs mean you're doing well and can continue what you're doing:

- You don't have new or worsening shortness of breath.
- You have very little swelling or no new swelling.
- Your weight is stable (no gain or loss).
- You have a normal activity level.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- You don't have chest pain or any other new symptoms.

Yellow zone



These signs and symptoms mean your condition may be getting worse and you should make some changes:

- You have trouble breathing when you're active.
- You have swelling in your feet or legs or have discomfort in your belly.
- You gain 2–3 lb (0.9–1.4 kg) in 24 hours, or 5 lb (2.3 kg) in a week. This amount may be more or less depending on your condition.
- You get tired easily.
- You have trouble sleeping.
- You have a dry cough.

If you have any of these symptoms:

- Contact your provider within the next day.
- Your provider may adjust your medicines.

Red zone



These signs and symptoms mean you should get medical help right away:

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

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Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- You have trouble breathing when resting or cannot lie flat and you need to raise your head to help you breathe.
- You have a dry cough that's getting worse.
- You have swelling or pain in your feet or legs or discomfort in your belly that's getting worse.
- You suddenly gain more than 2–3 lb (0.9–1.4 kg) in 24 hours, or more than 5 lb (2.3 kg) in a week. This amount may be more or less depending on your condition.
- You have trouble staying awake or you feel confused.
- You don't have an appetite.
- You have worsening sadness or depression.

These symptoms may be an emergency. Call 911 right away.

- **Do not wait to see if the symptoms will go away.**
- **Do not drive yourself to the hospital.**

Follow these instructions at home:

- Take medicines only as told.
- Eat a heart-healthy diet. Work with a dietitian to create an eating plan that's best for you.
- Weigh yourself each day. Your target weight is _____ lb (_____ kg).
 - 46 Call your provider if you gain more than _____ lb (_____ kg) in 24 hours, or more than _____ lb (_____ kg) in a week.
 - 46 Health care provider name: _____
 - 46 Health care provider phone number: _____

Where to find more information

- American Heart Association: heart.org

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Arkansas Heart Hospital

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MRN: 00213113

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DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Heart Failure: How to Manage

Heart failure is a long-term condition where your heart can't pump enough blood through your body. When this happens, parts of your body don't get the blood and oxygen they need.

There's no cure for heart failure, so it's important to take good care of yourself and follow the treatment plan you set with your health care provider. If you're living with heart failure, there are ways to help you manage the disease.

How to manage lifestyle changes

Living with heart failure requires you to make changes in your daily life. Your health care team will teach you about the changes you need to make. These changes can help improve your symptoms and lower your risk of going to the hospital. Work with your provider to create a plan for you.

Activity

- Ask your provider about cardiac rehabilitation programs. These programs include physical activity.
- If no cardiac rehab program is available, ask your provider what exercises are safe for you to do.
- Ask what things are safe for you to do at home. Ask when you can go back to work or school.
- Pace your daily activities and allow time for rest.

Managing stress

It's normal to have many emotions when you're told you have heart failure. You may have fear, sadness, and anger. If you need help coping with any of these emotions, let your provider know. Here are some ways to help yourself manage these emotions:

- Talk to your friends and family about your condition.
 - 46 They can give you support and guidance.
 - 46 Explain your symptoms to them. If comfortable, you can invite them to attend appointments with you.
- Join a support group for people with heart failure.
 - 46 Talking with others who have the same symptoms may give you new ways of coping with your disease and your emotions.
- Accept help from others. **Do not** be ashamed if you need help.
- Use stress management techniques. Try things like meditation, breathing exercises, or listening to relaxing music.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Conditions like depression and anxiety are common in people with heart failure. Pay attention to changes in your mood, emotions, and stress levels. Tell your provider if you have any of the following symptoms:

- Trouble sleeping or a change in your sleeping patterns.
- Feeling sad or depressed.
- Losing interest in activities you normally enjoy.
- Feeling irritable or crying for no reason.
- Often worrying about the future.

Work

If heart failure affects your ability to work, you may need to talk with your provider about making a plan for changes. This may include:

- Reducing your work hours.
- Finding tasks that require less effort.
- Planning rest periods during work hours.

Travel

- Talk with your provider if you plan to travel. There may be times when your provider suggests that you don't travel or you wait until your condition has improved.
- Bring your medicine and a list of your medicines. If you're traveling by public transportation (airplane, train, bus), keep your medicines with you in a carry-on bag.
- If you have special needs, contact the transportation company prior to traveling. This may include needs related to diet, oxygen, a wheelchair, a seating request, or help with luggage.
- Think about finding a medical facility in the area you'll be traveling to. Find out what your health insurance will cover.
- If you use oxygen, make sure you bring enough oxygen with you.
- If you have a battery-powered oxygen device, bring a fully charged extra battery with you.
- If you have an implanted device, bring a note from your provider and tell the security screening workers that you have the device. You may need to go through special screening.

Sexual activity

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation



- Ask your provider when it's safe for you to resume sexual activity.
- You may need to start slowly and increase intimacy over time.
- Regular exercise can benefit your sex life by building strength and endurance.

Sleep



If your condition affects your sleep, find ways to improve how well you sleep at night. These tips may help:

- Sleep lying on your side, or sleep with your head raised. Try:
 - 46 Raising the head of your bed.
 - 46 Using more than one pillow.
- Ask your provider about screening for sleep apnea.
- Try to go to sleep and wake up at the same times every day.
- Sleep in a dark, cool room.
- **Do not** exercise or eat for a few hours before bedtime.
- Plan rest periods during the day. But don't take long naps.

Where to find support

In addition to talking with your family or friends, consider talking with:

- A mental health professional or therapist.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- A member of your church, faith, or community group.

Other sources of support include:

- Local support groups. Ask your provider about groups near you.
- Online support groups, such as those found through:
 - 46 Heart Failure Society of America: hfsa.org
 - 46 American Heart Association: supportnetwork.heart.org
- Local community agencies or social agencies.
- A palliative care specialist. Palliative care can help manage symptoms, promote comfort, improve quality of life, and maintain dignity.

Where to find more information

To learn more, go to these websites:

- Centers for Disease Control and Prevention at cdc.gov. Then:
 - 46 Click Health Topics.
 - 46 Type "heart failure" in the search box.
- American Heart Association: heart.org
- National Heart, Lung, and Blood Institute: nhlbi.nih.gov

Contact a health care provider if:

- You have trouble breathing when you're active.
- You have swelling in your feet or legs.
- You gain 2–3 lb (0.9–1.4 kg) in 24 hours, or 5 lb (2.3 kg) in a week.
- You have a dry cough.
- You're not able to take part in your usual physical activities.
- You feel dizzy or light-headed when you stand up.
- You have trouble sleeping.
- You have a decrease in appetite.
- You have symptoms of depression or anxiety.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Get help right away if:

- You have trouble breathing when resting.
- You have trouble staying awake or you feel confused.
- You have chest pain.
- You faint.
- You have fast or irregular heartbeats.

These symptoms may be an emergency. Call 911 right away.

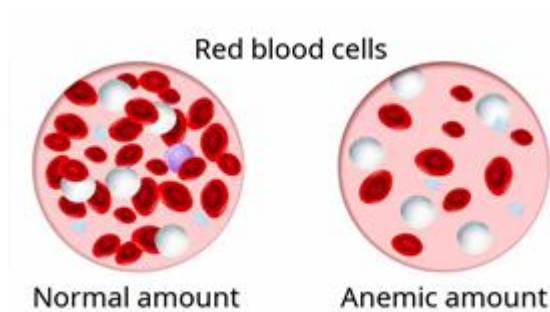
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Hematology

Anemia



Anemia is a condition in which there are not enough red blood cells or hemoglobin in the blood. Hemoglobin is a substance in red blood cells that carries oxygen.

When you do not have enough red blood cells or hemoglobin (are anemic), your body cannot get enough oxygen, and your organs may not work properly. As a result, you may feel very tired or have other problems.

What are the causes?

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Common causes of anemia include:

- Excessive bleeding. Anemia can be caused by excessive bleeding inside or outside the body, including bleeding from the intestines or from heavy menstrual periods in females.
- Poor nutrition.
- Long-lasting (chronic) kidney, thyroid, and liver disease.
- Bone marrow disorders, spleen problems, and blood disorders.
- Cancer and treatments for cancer.
- Human immunodeficiency virus (HIV) and acquired immunodeficiency syndrome (AIDS).
- Infections, medicines, and autoimmune disorders that destroy red blood cells.

What are the signs or symptoms?

Symptoms of this condition include:

- Minor weakness.
- Dizziness.
- Headache, or difficulties concentrating and sleeping.
- Heartbeats that feel irregular or faster than normal (palpitations).
- Shortness of breath, especially with exercise.
- Pale skin, lips, and nails, or cold hands and feet.
- Upset stomach (indigestion) and nausea.

Symptoms may occur suddenly or develop slowly. If your anemia is mild, you may not have symptoms.

How is this diagnosed?

This condition is diagnosed based on blood tests, your medical history, and a physical exam. In some cases, a test may be needed in which cells are removed from the soft tissue inside of a bone and looked at under a microscope (bone marrow biopsy). Your health care provider may also check your stool (feces) for blood and may do more testing to look for the cause of your bleeding.

Other tests may include:

- Imaging tests, such as a CT scan or MRI.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- A procedure to see inside your esophagus and stomach (endoscopy). The esophagus is the part of the body that moves food from your mouth to your stomach.
- A procedure to see inside your colon and rectum (colonoscopy).

How is this treated?

Treatment for this condition depends on the cause. If you continue to lose a lot of blood, you may need to be treated at a hospital. Treatment may include:

- Taking supplements of iron, vitamin B12, or folic acid.
- Taking a hormone medicine (erythropoietin) that can help to stimulate red blood cell growth.
- Receiving donated blood through an IV (blood transfusion). This may be needed if you lose a lot of blood.
- Making changes to your diet.
- Having surgery to remove your spleen.

Follow these instructions at home:

- Take over-the-counter and prescription medicines only as told by your health care provider.
- Take supplements only as told by your health care provider.
- Follow any diet instructions that you were given by your health care provider.
- Keep all follow-up visits. Your health care provider will want to recheck your blood tests.

Contact a health care provider if:

- You develop new bleeding anywhere in the body.
- You are very weak.

Get help right away if:

- You are short of breath.
- You have pain in your abdomen or chest.
- You are dizzy or feel faint.
- You have trouble concentrating.
- You have bloody stools, black stools, or tarry stools.
- You vomit repeatedly or you vomit up blood.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

These symptoms may be an emergency. Get help right away. Call 911.

- **Do not wait to see if the symptoms will go away.**
- **Do not drive yourself to the hospital.**

Summary

- Anemia is a condition in which you do not have enough red blood cells or enough of a substance in your red blood cells that carries oxygen.
- Symptoms may occur suddenly or develop slowly.
- If your anemia is mild, you may not have symptoms.
- This condition is diagnosed with blood tests, a medical history, and a physical exam. Other tests may be needed.
- Treatment for this condition depends on the cause of the anemia.

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sacubitril and valsartan

(sak UE bi tril and val SAR tan)

Entresto, Entresto Sprinkle

What is the most important information I should know about sacubitril and valsartan?

Do not use if you are pregnant. Stop using this medicine and tell your doctor right away if you become pregnant.

If you have diabetes, do not take sacubitril and valsartan with any medication that contains aliskiren (a blood pressure medicine).

Tell your doctor about all your other medicines. **Some drugs should not be used with sacubitril and valsartan.**

What is sacubitril and valsartan?

Sacubitril and valsartan is a combination medicine that is used in adults with chronic heart failure. This medicine lowers the risk of being hospitalized when symptoms get worse and reduces the risk of death from heart failure.

Sacubitril and valsartan is also used to treat symptomatic heart failure in children who are at least 1 year old.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Sacubitril and valsartan may also be used for purposes not listed in this medication guide.

What should I discuss with my healthcare provider before taking sacubitril and valsartan?

You should not use this medicine if you are allergic to sacubitril or valsartan or if you have or have ever had a severe allergic reaction to similar medicines such as:

- an ACE inhibitor--benazepril, captopril, enalapril, fosinopril, lisinopril, moexipril, perindopril, quinapril, ramipril, trandolapril; or
- an ARB--azilsartan, candesartan, eprosartan, irbesartan, losartan, olmesartan, telmisartan, valsartan.

You should not take sacubitril and valsartan within 36 hours before or after you have taken any ACE inhibitor medication.

If you have diabetes, do not take sacubitril and valsartan with any medication that contains aliskiren (a blood pressure medicine).

You may also need to avoid taking sacubitril and valsartan with aliskiren **if you have kidney disease**.

Tell your doctor if you have or have ever had:

- diabetes;
- low blood pressure;
- low levels of aldosterone (hypoaldosteronism);
- hereditary angioedema;
- if you are dehydrated;
- if you are on a low-salt-diet;
- if you are on a high-potassium diet; or
- severe liver or kidney disease.

Stop using this medicine and tell your doctor right away if you become pregnant. Sacubitril and valsartan can cause injury or death to the unborn baby if you use the medicine during your second or third trimester.

Ask a doctor if it is safe to breastfeed while using this medicine.

How should I take sacubitril and valsartan?

Follow all directions on your prescription label and read all medication guides or instruction sheets. Your doctor may occasionally change your dose. Use the medicine exactly as directed.

Your dose needs may change if you switch to a different strength or form of this medicine. **Avoid medication errors by using exactly as directed on the label or as prescribed by your doctor.**

Sacubitril and valsartan is usually taken twice per day with or without food. You may need to take more than one capsule or tablet to receive a full dose. Follow your doctor's instructions.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Doses are based on weight in children and teenagers. Your child's dose may change if the child gains or loses weight.

Read and carefully follow the Instructions for Use provided with your medicine. Ask your doctor or pharmacist if you don't understand these instructions.

If you are unable to swallow a tablet whole, a pharmacist can make an oral suspension (liquid). Tell the doctor if the person taking this medicine has trouble swallowing the tablet.

Shake the **oral suspension** (liquid). Measure a dose with the supplied measuring device (not a kitchen spoon).

The **oral pellets** are inside the capsule. Do not swallow the capsule whole, and do not chew and crush the **oral pellets** inside the capsule. The capsule must be opened before taking the **oral pellets**. Do not swallow the empty capsule shell after opening.

To take the **oral pellets**, open the capsule and sprinkle the contents with 1 to 2 teaspoons of a soft food, such as applesauce, yogurt, or pudding. Swallow the mixture right away after adding the pellets.

Do not give the **oral pellets** through a feeding tube. The pellets may clog the tube.

Your doctor will need to check your progress on a regular basis. Your blood and kidney function may need to be tested often.

Store at room temperature away from moisture and heat. Throw away any **oral suspension** not used within 15 days after it was mixed. **Do not keep the oral suspension in a refrigerator.**

What happens if I miss a dose?

Take the medicine as soon as you can, but skip the missed dose if it is almost time for your next dose. **Do not** take two doses at one time.

What happens if I overdose?

Seek emergency medical attention or call the Poison Help line at 1-800-222-1222.

What should I avoid while taking sacubitril and valsartan?

Do not take potassium supplements or use salt substitutes, unless your doctor has told you to.

What are the possible side effects of sacubitril and valsartan?

Get emergency medical help if you have **signs of an allergic reaction**: hives, difficult breathing, swelling of your face, lips, tongue, or throat.

Call your doctor at once if you have:

- feeling like you might pass out;
- **high blood potassium**--nausea, weakness, tingly feeling, chest pain, irregular heartbeats, loss of movement; or
- **kidney problems**--swelling, urinating less, feeling tired or short of breath.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

Date of Service: 6/15/2026

FIN: 3670575

Provider: Paixao,MD,Andre

DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Common side effects may include:

- kidney problems;
- high potassium levels in your blood;
- dizziness, feeling like you might pass out; or
- cough.

This is not a complete list of side effects and others may occur. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

What other drugs will affect sacubitril and valsartan?

Tell your doctor about all your other medicines, especially:

- lithium;
- a diuretic or 'water pill' that may increase blood potassium such as spironolactone, triamterene, or amiloride;
- heart or blood pressure medication;
- medicine or mineral supplements that contain potassium; or
- **NSAIDs (nonsteroidal anti-inflammatory drugs)**--aspirin, ibuprofen (Advil, Motrin), naproxen (Aleve), celecoxib, diclofenac, indomethacin, meloxicam, and others.

This list is not complete. Other drugs may affect sacubitril and valsartan, including prescription and over-the-counter medicines, vitamins, and herbal products. Not all possible drug interactions are listed here.

Where can I get more information?

Your doctor or pharmacist can provide more information about sacubitril and valsartan.

Remember, keep this and all other medicines out of the reach of children, never share your medicines with others, and use this medication only for the indication prescribed.

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Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

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dapagliflozin

(DAP a gli FLOE zin)

Farxiga

What is the most important information I should know about dapagliflozin?

Tell your doctor if you are sick with vomiting or diarrhea, or if you eat or drink less than usual.

Dapagliflozin can cause serious infections around the penis or vagina. Get medical help right away if you have burning, itching, odor, discharge, pain, tenderness, redness or swelling of the genital or rectal area, fever, or if you don't feel well.

What is dapagliflozin?

Dapagliflozin is used with diet and exercise to improve blood sugar control in adults with type 2 diabetes mellitus (**not for type 1 diabetes**).

Dapagliflozin is also used to lower the risk of needing to be in a hospital for heart failure in adults with type 2 diabetes who also have heart disease.

Dapagliflozin is also used in adults to lower the risk of dying or needing to be in a hospital for heart failure when your heart cannot pump blood properly.

Dapagliflozin is also used to reduce the risk of end-stage kidney disease and hospitalization or death from heart problems in adults who also have kidney problems caused by type 2 diabetes.

Dapagliflozin may also be used for purposes not listed in this medication guide.

What should I discuss with my healthcare provider before taking dapagliflozin?

You should not use dapagliflozin if you are allergic to it, or if you have:

- severe kidney disease (or if you are on dialysis); or
- diabetic ketoacidosis (call your doctor for treatment).

Tell your doctor if you have ever had:

- polycystic kidney disease;
- liver disease;
- bladder infections or other urination problems;
- problems with your pancreas, including surgery;
- alcoholism, or if you currently drink large amounts of alcohol; or

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- if you are on a low salt diet.

Follow your doctor's instructions about using this medicine **if you are pregnant or you become pregnant**. Controlling diabetes is very important during pregnancy.

You should not use dapagliflozin during the second or third trimester of pregnancy.

Do not breastfeed.

How should I take dapagliflozin?

Follow all directions on your prescription label and read all medication guides or instruction sheets. Your doctor may occasionally change your dose. Use the medicine exactly as directed.

You may take dapagliflozin with or without food.

Your blood sugar will need to be checked often, and you may also need to test the level of ketones in your urine. **Dapagliflozin can cause life-threatening ketoacidosis (too much acid in the blood).** Even if your blood sugar is normal, contact your doctor if a urine test shows that you have high ketones in the urine.

Blood sugar can be affected by stress, illness, surgery, exercise, alcohol use, or skipping meals.

Low blood sugar (**hypoglycemia**) can make you feel very hungry, dizzy, irritable, or shaky. To quickly treat hypoglycemia, eat or drink hard candy, crackers, raisins, fruit juice, or non-diet soda. Your doctor may prescribe glucagon injection in case of severe hypoglycemia.

You may get dehydrated during prolonged illness. Call your doctor if you are sick with vomiting or diarrhea, or if you eat or drink less than usual.

This medicine can affect the results of certain medical tests. Tell any doctor who treats you that you are using dapagliflozin.

Your treatment may also include diet, exercise, weight control, and special medical care.

Store at room temperature away from moisture and heat.

What happens if I miss a dose?

Take the medicine as soon as you can, but skip the missed dose if it is almost time for your next dose. **Do not** take two doses at one time.

What happens if I overdose?

Seek emergency medical attention or call the Poison Help line at 1-800-222-1222.

What should I avoid while taking dapagliflozin?

Avoid drinking alcohol.

Avoid getting up too fast from a sitting or lying position, or you may feel dizzy.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

What are the possible side effects of dapagliflozin?

Get emergency medical help if you have **signs of an allergic reaction**: hives; difficult breathing; swelling of your face, lips, tongue, or throat.

Seek medical attention right away if you have signs of a genital infection (penis or vagina): burning, itching, odor, discharge, pain, tenderness, redness or swelling of the genital or rectal area, fever, not feeling well. **These symptoms may get worse quickly.**

Call your doctor at once if you have:

- **dehydration**--dizziness, confusion, feeling very thirsty, less urination;
- **kidney problems**--little or no urination, swelling in your feet or ankles, feeling tired or short of breath;
- **ketoacidosis (too much acid in the blood)**--nausea, vomiting, stomach pain, confusion, unusual drowsiness, or trouble breathing; or
- **signs of a bladder infection**--pain or burning when you urinate, increased urination, blood in your urine, fever, pain in your pelvis or back.

Some side effects may be more likely to occur in older adults.

Common side effects may include:

- genital yeast infection;
- urinating more than usual; or
- sore throat and runny or stuffy nose.

This is not a complete list of side effects and others may occur. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

What other drugs will affect dapagliflozin?

Other drugs may increase or decrease the effects of dapagliflozin on lowering your blood sugar. Tell your doctor about all your current medicines and any you start or stop using, especially:

- insulin or other oral diabetes medicines;
- a diuretic or 'water pill'; or
- lithium.

This list is not complete. Other drugs may affect dapagliflozin, including prescription and over-the-counter medicines, vitamins, and herbal products. Not all possible drug interactions are listed here.

Where can I get more information?

Your doctor or pharmacist can provide more information about dapagliflozin.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

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FIN: 3670575

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Documentation

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spironolactone

(spir ON oh LAK tone)

Aldactone, CaroSpir

What is the most important information I should know about spironolactone?

You should not use spironolactone if you have Addison's disease, high levels of potassium in your blood, if you are unable to urinate, or if you are also taking eplerenone.

What is spironolactone?

Spironolactone is a potassium-sparing diuretic (water pill) that prevents your body from absorbing too much salt and keeps your potassium levels from getting too low.

Spironolactone is used to treat heart failure, high blood pressure (hypertension), or hypokalemia (low potassium levels in the blood).

Spironolactone also treats fluid retention (edema) in people with congestive heart failure, cirrhosis of the liver, or a kidney disorder called nephrotic syndrome.

Spironolactone is also used to diagnose or treat a condition in which you have too much aldosterone in your body. Aldosterone is a hormone produced by your adrenal glands to help regulate the salt and water balance in your body.

Spironolactone may also be used for purposes not listed in this medication guide.

What should I discuss with my healthcare provider before taking spironolactone?

You should not use spironolactone if you are allergic to it, or if you have:

- Addison's disease (an adrenal gland disorder);

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

Date of Service: 6/15/2026

FIN: 3670575

Provider: Paixao,MD,Andre

DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

- high levels of potassium in your blood (hyperkalemia);
- if you are unable to urinate; or
- if you are also taking eplerenone.

Tell your doctor if you have ever had:

- an electrolyte imbalance (such as low levels of calcium, magnesium, or sodium in your blood);
- kidney disease;
- liver disease; or
- heart disease.

Tell your doctor if you are pregnant or plan to become pregnant. Having congestive heart failure, cirrhosis, or uncontrolled high blood pressure during pregnancy may lead to medical problems in the mother or the baby. **Your doctor should decide whether you take spironolactone if you are pregnant.**

It may not be safe to breastfeed while using this medicine. Ask your doctor about any risk.

How should I take spironolactone?

Follow all directions on your prescription label and read all medication guides or instruction sheets. Your doctor may occasionally change your dose. Use the medicine exactly as directed.

Do not share this medicine with another person, even if they have the same symptoms you have.

You may take spironolactone with or without food, but take it the same way each time.

You will need frequent medical tests.

This medicine can affect the results of certain medical tests. Tell any doctor who treats you that you are using spironolactone.

If you need surgery, tell your surgeon you currently use this medicine. You may need to stop for a short time.

If you are being treated for high blood pressure, **keep using this medication even if you feel well**. High blood pressure often has no symptoms. You may need to use blood pressure medication for the rest of your life.

Store at room temperature away from heat, light, and moisture.

What happens if I miss a dose?

Take the medicine as soon as you can, but skip the missed dose if it is almost time for your next dose. **Do not** take two doses at one time.

What happens if I overdose?

Seek emergency medical attention or call the Poison Help line at 1-800-222-1222.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

What should I avoid while taking spironolactone?

Drinking alcohol can increase certain side effects.

Do not use potassium supplements or salt substitutes, unless your doctor has told you to.

Avoid a diet high in salt. Too much salt will cause your body to retain water and can make this medication less effective.

Avoid driving or hazardous activity until you know how this medicine will affect you. Your reactions could be impaired. Avoid getting up too fast from a sitting or lying position, or you may feel dizzy.

What are the possible side effects of spironolactone?

Get emergency medical help if you have **signs of an allergic reaction:** hives; difficulty breathing; swelling of your face, lips, tongue, or throat.

Call your doctor at once if you have:

- a light-headed feeling, like you might pass out;
- little or no urination;
- **high potassium level**--nausea, weakness, tingly feeling, chest pain, irregular heartbeats, loss of movement; o
- **signs of other electrolyte imbalances**--increased thirst or urination, confusion, vomiting, muscle pain, slurred speech, severe weakness, numbness, loss of coordination, feeling unsteady.

Common side effects may include:

- breast swelling or tenderness.

This is not a complete list of side effects and others may occur. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

What other drugs will affect spironolactone?

Using spironolactone with other drugs that make you dizzy can worsen this effect. Ask your doctor before using opioid medication, a sleeping pill, a muscle relaxer, or medicine for anxiety, depression, or seizures.

Tell your doctor about all your other medicines, especially:

- colchicine;
- digoxin;
- lithium;
- loperamide;
- trimethoprim;

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

- heart or blood pressure medicine (especially another diuretic);
- medicine to prevent a blood clot; or
- **NSAIDs (nonsteroidal anti-inflammatory drugs)**--aspirin, ibuprofen (Advil, Motrin), naproxen (Aleve), celecoxib, diclofenac, indomethacin, meloxicam, and others.

This list is not complete. Other drugs may affect spironolactone, including prescription and over-the-counter medicines, vitamins, and herbal products. Not all possible drug interactions are listed here.

Where can I get more information?

Your pharmacist can provide more information about spironolactone.

Remember, keep this and all other medicines out of the reach of children, never share your medicines with others, and use this medication only for the indication prescribed.

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amlodipine

(am LOE di peen)

Katerzia, Norliqva, Norvasc

What is the most important information I should know about amlodipine?

Use only as directed. Tell your doctor if you use other medicines or have other medical conditions or allergies.

What is amlodipine?

Amlodipine is used in adults alone or in combination with other medicines to treat chest pain (angina) and other conditions caused by coronary artery disease.

Amlodipine is also used alone or in combination with other medicines to treat high blood pressure (hypertension) in adults and children at least 6 years old. Lowering blood pressure may lower your risk of a stroke or heart attack.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Amlodipine may also be used for purposes not listed in this medication guide.

What should I discuss with my healthcare provider before taking amlodipine?

You should not take amlodipine if you are allergic to it.

Tell your doctor if you have ever had:

- severe narrowing of a certain valve in your heart (aortic stenosis), or other heart problems;
- coronary artery disease (clogged arteries); or
- liver disease.

It is not known if amlodipine will harm an unborn baby. However, having high blood pressure during pregnancy may cause complications or medical problems in both mother and baby. The benefit of treating hypertension may outweigh any risks to the baby.

Tell your doctor if you are breastfeeding.

How should I take amlodipine?

Follow all directions on your prescription label and read all medication guides or instruction sheets. Your doctor may occasionally change your dose. Use the medicine exactly as directed.

Amlodipine is usually taken once per day.

Take the medicine at the same time each day, with or without food.

Shake the **oral suspension** (liquid). Measure a dose with the supplied measuring device (not a kitchen spoon).

Your blood pressure will need to be checked often.

If you have high blood pressure, **keep using this medicine even if you feel well**. High blood pressure often has no symptoms.

Do not change your doses or stop taking any of your medications without your doctor's advice.

Your treatment may also include diet, exercise, weight control, and blood tests.

Store amlodipine **tablets** and **oral solution** (liquid) at room temperature away from moisture, heat, and light.

Store amlodipine **oral suspension** in the refrigerator, do not freeze.

What happens if I miss a dose?

Take the medicine as soon as you can, but skip the missed dose if you are more than 12 hours late for the dose. **Do not** take two doses at one time.

What happens if I overdose?

Seek emergency medical attention or call the Poison Help line at 1-800-222-1222.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

Date of Service: 6/15/2026

FIN: 3670575

Provider: Paixao,MD,Andre

DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Overdose symptoms may include rapid heartbeats, redness or warmth in your arms or legs, or fainting.

What should I avoid while taking amlodipine?

Avoid getting up too fast from a sitting or lying position, or you may feel dizzy.

What are the possible side effects of amlodipine?

Get emergency medical help if you have **signs of an allergic reaction**: hives; difficulty breathing; swelling of your face, lips, tongue, or throat.

In rare cases, when you first start taking amlodipine, your chest pain may get worse or you could have a heart attack. **Seek emergency medical attention or call your doctor right away if you have symptoms such as:** chest pain or pressure, pain spreading to your jaw or shoulder, nausea, sweating.

Call your doctor at once if you have:

- worsening chest pain; or
- a light-headed feeling, like you might pass out.

Common side effects may include:

- dizziness, drowsiness;
- swelling of your legs or ankles;
- irregular heartbeat;
- pounding heartbeats or fluttering in your chest;
- muscle stiffness;
- uncontrolled muscle movements;
- feeling tired;
- stomach pain, nausea; or
- flushing (sudden warmth, redness, or tingly feeling).

This is not a complete list of side effects and others may occur. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

What other drugs will affect amlodipine?

Sometimes it is not safe to use certain medicines at the same time. Some drugs can affect your blood levels of other drugs you use, which may increase side effects or make the medicines less effective.

Other drugs may affect amlodipine, including prescription and over-the-counter medicines, vitamins, and herbal products. Tell your doctor about all other medicines you use.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

DOB/Age/Sex: 5/1/1937 89 years Female

Date of Service: 6/15/2026

Provider: Paixao,MD,Andre

Documentation

Where can I get more information?

Your doctor or pharmacist can provide more information about amlodipine.

Remember, keep this and all other medicines out of the reach of children, never share your medicines with others, and use this medication only for the indication prescribed.

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torsemide (oral/injection)

(TOR se mide)

Soaanz

What is the most important information I should know about torsemide?

You should not use this medicine if you are allergic to sulfa drugs or if you are unable to urinate.

What is torsemide?

Torsemide is a loop diuretic (water pill) that is used to treat fluid retention (edema) in people with heart failure, liver disease, or kidney disease.

Torsemide is also used to treat high blood pressure (hypertension). Lowering blood pressure may lower your risk of a stroke or heart attack.

Torsemide may also be used for purposes not listed in this medication guide.

What should I discuss with my healthcare provider before taking torsemide?

You should not use torsemide if you are allergic to it, or if:

- you are unable to urinate; or
- you are allergic to an oral diabetes medicine.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

Date of Service: 6/15/2026

FIN: 3670575

Provider: Paixao,MD,Andre

DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Tell your doctor if you have ever had:

- heart disease;
- kidney problems (unless you are taking torsemide for edema related to kidney disease);
- liver problems such as cirrhosis or hepatitis (unless you are taking torsemide for edema related to liver disease);
- gout;
- an allergy to povidone (sometimes contained in iodine, Betadine); or
- if you are on a low-salt diet.

If you need to have any type of x-ray or CT scan using a dye that is injected into your veins, tell your caregivers that you currently take torsemide.

Tell your doctor if you are pregnant or plan to become pregnant. It is not known whether torsemide will harm an unborn baby. However, having high blood pressure during pregnancy may cause complications such as diabetes or eclampsia (dangerously high blood pressure that can lead to medical problems in both mother and baby). The benefit of treating hypertension may outweigh any risks to the baby.

It may not be safe to breastfeed while using this medicine. Ask your doctor about any risk.

Torsemide is not approved for use by anyone younger than 18 years old.

How should I take torsemide?

Follow all directions on your prescription label and read all medication guides or instruction sheets. Your doctor may occasionally change your dose. Use the medicine exactly as directed.

Torsemide **oral** is taken by mouth.

Torsemide **injection** is given as an infusion into a vein. A healthcare provider may teach you how to properly use the medication by yourself. Torsemide **injection** is given either as a slow infusion over 2 minutes, or as a continuous (around the clock) infusion.

If you have cirrhosis or other liver problems, you may receive your first **injection** in a hospital or clinic setting to quickly treat any serious side effects.

Read and carefully follow any Instructions for Use provided with your medicine. Ask your doctor or pharmacist if you don't understand all instructions.

Prepare an injection only when you are ready to give it. **Do not use if the medicine has changed colors or has particles in it.** Call your pharmacist for new medicine.

You may take torsemide with or without food.

Your blood pressure will need to be checked often, and you may need other medical tests. Your blood and urine may both be tested if you have been vomiting or are dehydrated.

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

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Date of Service: 6/15/2026

FIN: 3670575

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DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Some things can cause your blood pressure to get too low. This includes vomiting, diarrhea, or heavy sweating. Call your doctor if you are sick with vomiting or diarrhea.

Your condition may be treated with a combination of drugs. Use all medications as directed by your doctor. **Do not change your doses or medication schedule without your doctor's advice.**

If you have high blood pressure, **keep taking torsemide oral even if you feel well.** High blood pressure often has no symptoms. You may need to use blood pressure medicine for the rest of your life.

Store at room temperature away from moisture and heat.

What happens if I miss a dose?

Take the medicine as soon as you can, but skip the missed dose if it is almost time for your next dose. **Do not** take two doses at one time.

Call your doctor for instructions if you miss a dose of torsemide **injection**.

What happens if I overdose?

Seek emergency medical attention or call the Poison Help line at 1-800-222-1222.

Overdose symptoms may include dehydration, an electrolyte imbalance (such as low levels of potassium, magnesium, or sodium in your blood), or low blood pressure and fainting.

What should I avoid while taking torsemide?

Avoid becoming overheated or dehydrated during exercise, in hot weather, or by not drinking enough fluids. Follow your doctor's instructions about the type and amount of liquids you should drink. In some cases, drinking too much liquid can be as unsafe as not drinking enough.

What are the possible side effects of torsemide

Get emergency medical help if you have **signs of an allergic reaction**: hives; difficult breathing; swelling of your face, lips, tongue, or throat.

Call your doctor right away if you have:

- a light-headed feeling, like you might pass out;
- ringing in your ears, sudden hearing loss;
- **kidney problems**--little or no urination, painful or difficult urination, swelling in your feet or ankles, feeling tired or short of breath;
- **low magnesium level**--dizziness, irregular heartbeats, feeling jittery, muscle cramps, muscle spasms, cough or choking feeling;
- **low potassium level**--leg cramps, constipation, irregular heartbeats, fluttering in your chest, increased thirst or urination, numbness or tingling, muscle weakness or limp feeling; or

Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

FIN: 3670575

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Provider: Paixao,MD,Andre

Documentation

- **low sodium level**--headache, confusion, slurred speech, severe weakness, vomiting, loss of coordination, feeling unsteady.

Common side effects may include:

- increased urination; or
- headache.

This is not a complete list of side effects and others may occur. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

What other drugs will affect torsemide?

Tell your doctor about all your current medicines. Many drugs can affect torsemide, especially:

- any other diuretic medicine;
- cholestyramine, lithium, phenytoin, probenecid, rifampin, warfarin (Coumadin, Jantoven);
- an intravenous (IV) antibiotic medicine;
- **antifungal medicine**--fluconazole, miconazole;
- **heart or blood pressure medicine**--amiodarone, dronedarone, enalapril, irbesartan, lisinopril, losartan, olmesartan, valsartan, and others;
- **NSAIDs (nonsteroidal anti-inflammatory drugs)**--ibuprofen (Advil, Motrin), naproxen (Aleve), celecoxib, diclofenac, indomethacin, meloxicam, and others;
- **salicylates**--aspirin, Nuprin Backache Caplet, Kaopectate, KneeRelief, Pamprin Cramp Formula, Pepto-Bismol, Tricosal, Trilisate, and others or
- **steroid medicine**--dexamethasone, oxandrolone, prednisone, and others.

This list is not complete and many other drugs may affect torsemide. This includes prescription and over-the-counter medicines, vitamins, and herbal products. Not all possible drug interactions are listed here.

Where can I get more information?

Your pharmacist can provide more information about torsemide.

Remember, keep this and all other medicines out of the reach of children, never share your medicines with others, and use this medication only for the indication prescribed.

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Arkansas Heart Hospital

Patient Name: MONSON, BERDIA T

MRN: 00213113

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